



GLOBAL ROBOTICS & INNOVATION CHAMPIONSHIP

IntelliScript BD

The Safety-First Clinical Co-Pilot

"A nationwide digital safety net protecting patients from fatal prescription errors and counterfeit drugs."

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The Critical Failures of the Status Quo

1. Clinical Blindness

Prescribing in a data vacuum without verified access to a patient's active medication list or allergy history.

2. The Handwritten Hazard

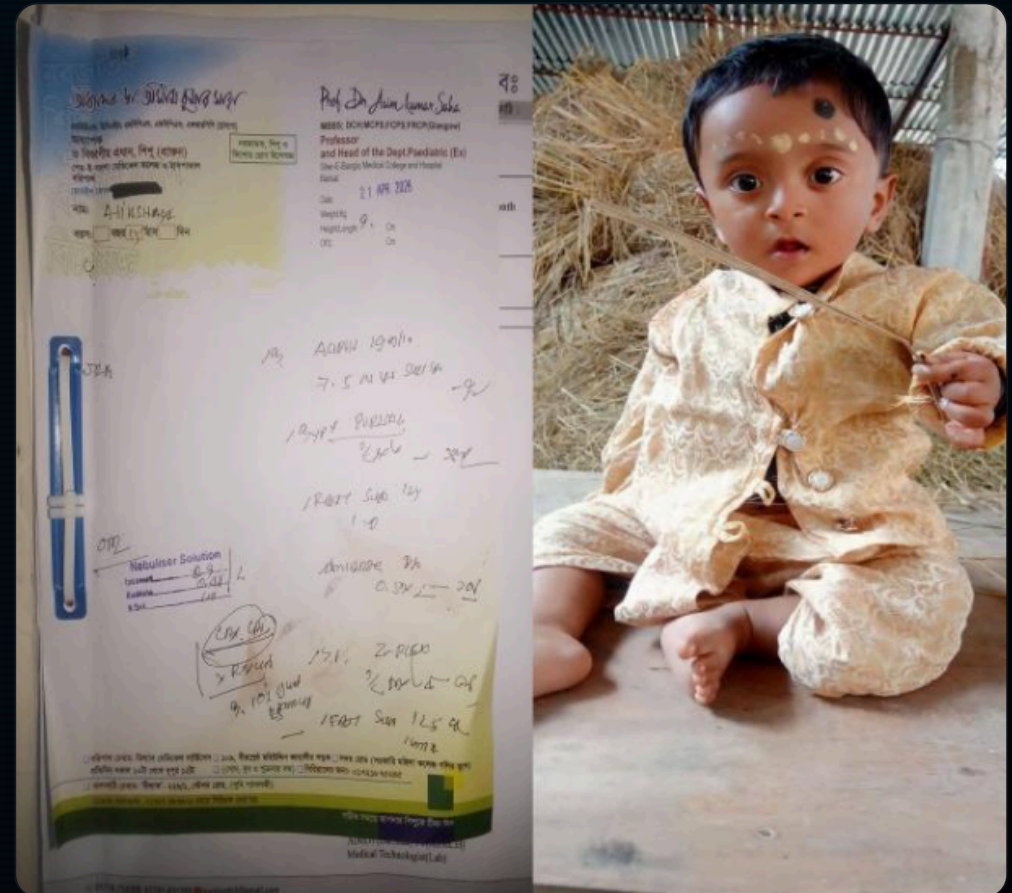
Illegible penmanship forces pharmacists to "guess" dosages (e.g. 10mg vs 70mg), leading to systemic toxicity before the first dose is finished.

3. Lethal Drug-Drug Interactions (DDI)

The current paper system lacks a 'Real-Time Interceptor,' allowing doctors to accidentally prescribe conflicting medications that cause systemic organ failure.

4. Unnecessary Testing

Lost paper records force doctors to re-order exact same tests (X-rays, ECGs), creating a massive financial drain on vulnerable families.



THE REALITY OF PAPER

Solution Overview

"IntelliScript BD is a **Sovereign Health Infrastructure**—an offline-first digital clinical pad designed to replace the dangerous paper-based status quo."

For the Doctor

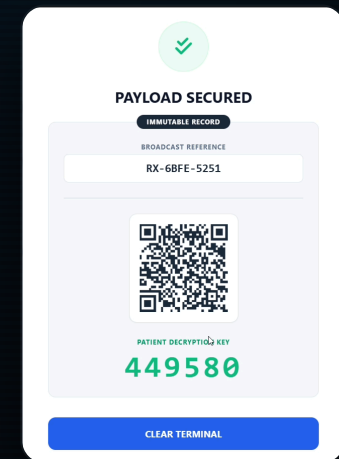
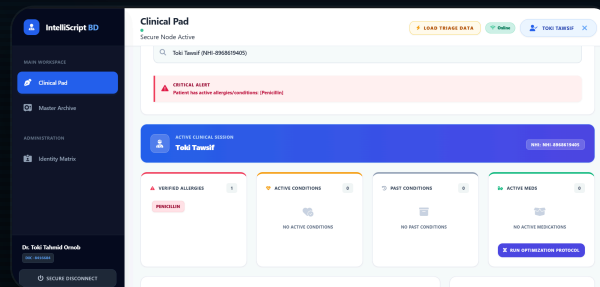
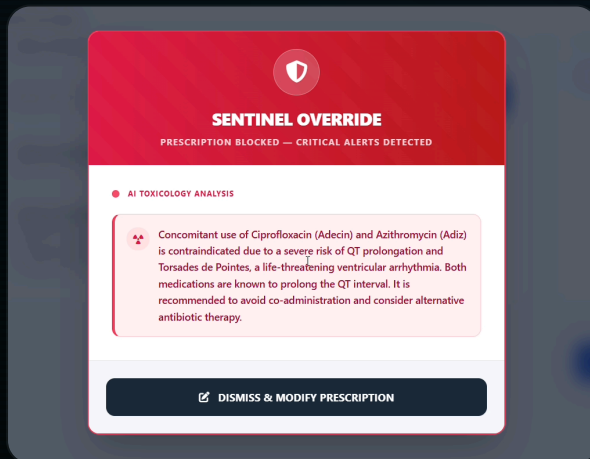
Active Interception. Acts as a digital safety net that physically prevents lethal prescribing errors in real-time.

For the Patient

Identity Permanence. Ensures medical history and allergies are never lost, regardless of where they seek care.

For the Pharmacist

Cryptographic Certainty. Eliminates handwriting guesswork with a verified, tamper-proof digital handshake.



How It Works: Zero-Trust Ecosystem



1. Authorize

Patient scans phone to grant doctor temporary, read-only access to their Universal Health Profile.



2. Prescribe

Doctor writes script on offline HTML5 pad. AI Sentinel cross-references data to block lethal interactions instantly.



3. Hash

System generates unbreakable SHA-256 QR code. Patient scans to store verified payload in Sovereign Wallet.



4. Dispense

Pharmacist scans patient's phone. Device-to-device decryption verifies authenticity, preventing counterfeit drugs.

Why Now? The Burning Platform

March 2026 (Sunamganj): 3-year-old poisoned by wrong medication due to doctor's handwriting misinterpretation. Heavy fines issued to pharmacy.

Nov 2025 (Noakhali): 4-year-old died within an hour of receiving a lethal injection administered without a verified medical history.

May 2026 (Jhalokathi): 5-year-old child died this month due to a high-dose antibiotic error ("Prescription Blindness").

The Forensic Data

Bogura Study (Pharmaceutical Journal): 366 dangerous interactions found in just 200 prescriptions; 12% were potentially lethal.

Clinical Gap: 96.7% of district hospital paper records lack critical previous diagnosis data, forcing 'Guess-work Medicine'.

Cardiac Risk: 56% to 60% of patient prescriptions, especially in cardiac care, contain at least one potential drug-drug interaction (Source: pharmascigro).

Regulatory Push: DNCRP is aggressively fining errors. Clinics are desperate for a tool that protects both patient lives and provider licenses.

Target Users & National Scale

TAM: 170 Million

Total Nation

Healthcare is universal. Foundational digital identity ceiling is the entire population.

SAM: 120+ Million

Mobile Users

Realistic digital-ready population who can interact with Sovereign Wallets and QR hashes.

SOM: 30 Million & 4k+ Clinics

Execution Target

Chronic care patients (highest DDI risk) and District Sadar hospitals lacking cloud infrastructure.

Business Model & National Impact

Tier 1: Clinical Land-Grab

Free-to-use for Doctors and Clinics. We eliminate financial friction to capture the network and establish the national standard.

Tier 2: Sovereign Safety Fee

10 BDT per digital prescription or 100 BDT for a 6-month pass. "Cup of tea" pricing generates massive B2C revenue at scale.

Tier 3: Enterprise Verification

B2B API Licensing for Pharma Giants (Square, Beximco) to provide "Authenticity Portals" preventing counterfeit losses.

Sustainability Proof (2024-2025 Data)

- Pharma market projected to surpass **\$6B by 2025**.
- NCDs account for **67% of total deaths** in BD.
- **30M+ citizens** live with chronic conditions requiring consistent, safe management.

The Inclusion Failsafe: Villagers without smartphones are served via SMS OTP to feature phones, managed by hospital desks. 100% coverage.

Competitive Advantage: The Active Moat

FEATURE	MANUAL PAPER	STANDARD E-HEALTH	INTELLISCRIP BD
Safety Logic	Zero (Memory)	Passive (Data Entry)	Active AI Interception
Data Integrity	High Forgery Risk	Basic DB Entry	Cryptographic Hashing
Clinical Speed	Instant (Unsafe)	Slow / Complex	Engineered UX Speed
Accessibility	100% (Universal)	20% (High-End Only)	Sovereign & OTP Access

Interception vs. Documentation: Competitors record what happened after the fact. Our AI Sentinel is a "Hard-Stop" that prevents the error from ever leaving the desk.

Sovereign vs. Siloed: Hospital databases trap your data. We put the cryptographic truth in the patient's hands, making it portable across the entire nation.

Cryptographic Truth vs. PDFs: Turn forgeable prescription PDFs into immutable SHA-256 hashes, ensuring what the doctor wrote is exactly what the pharmacist sees.

Mechanical Speed: Designed with a mechanical engineering mindset, our UI is stripped of friction, ensuring digital prescribing is as fast as handwriting but 100% safer.

Validation & Unfair Advantage

Proving demand, proving the moat, and proving why standard cloud-based E-Health systems cannot compete in this clinical reality.

N=22 SURVEY
VALIDATED

BLOCK 1

Why we beat HMS

The Unfair Advantage

Investors ask why hospitals will not just buy a standard cloud software package. The answer is infrastructure fit.

01

Zero Deployment Cost. Existing systems require expensive servers and high-speed internet. We run on existing legacy hardware.

02

100% Offline Persistence. Cloud HMS fail during outages. Our HTML5 architecture survives 6-hour load-shedding windows.

03

Active vs. Passive. Standard systems document data. AI Sentinel actively blocks lethal errors before release.

BLOCK 2

Why hospitals must buy

Economic & Legal Catalyst

The buying trigger is not only innovation. It is fear of fines, unnecessary cost, and malpractice exposure.

01

Regulatory Hammer. DNCRP is fining pharmacies and clinics for misreading illegible scripts. We eliminate this liability.

02

Financial Drain. 96.7% of history gaps force redundant diagnostic testing on vulnerable patients.

03

Clinical Liability. 56–60% of cardiac prescriptions have DDI risks. Doctors need a hard-stop safety layer.

BLOCK 3

Offline system demand

Quantitative Market Readiness

100%



Fear of Counterfeits

Expired or fake medicine concern

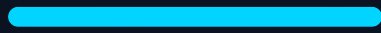
86.4%



Blind Allergy Risk

Emergency allergy gap recognized

100%



Offline Reliability

Cloud-only competitors are misfit

Investor read: market readiness is tied to trust, emergency safety, and offline reliability — exactly where our product is strongest.

BLOCK 4

In the trenches

Execution & Hustle Proof



Clinical Authority



User Empathy



Workflow Integration

Investor Signal: We are not just coders building theoretical software in a dorm room. We are validating the architecture with doctors, patients, and the medical supply chain.

Roadmap: Path to National Safety



M 1-2: BUILD

Map AI logic to top 500 drugs.
Finalize HTML5 localStorage for
6+ hr offline persistence.
Complete SMS OTP Help-Desk.



M 3: TEST

Hardware Stress Test (SHA-256
generation < 2s on 10yo PCs).
Independent Red-Team audit.
DGDA documentation.



M 4: PILOT

Launch 30-day "Live Beta" at
District Sadar Hospital. Track full
Doctor-Patient-Pharmacy loop
and error reduction.



M 5-6: SCALE

Refine UX speed. Present a2i
integration for national stack.
Secure B2B Pharma supply-
chain partnerships.

The Engineering Powerhouse



Toki Tahmid Ornob

SYSTEMS ARCHITECT

Mechanical Engineering core. Directs the "Mechanical-to-Digital" workflow, ensuring the software remains compatible with the rigorous, fast-paced reality of clinical environments.



Tabasum Aktar

SECURITY ARCHITECT

CSE specialization. Leads the development of the AI Sentinel, SHA-256 encryption logic, and the un-crashable Offline HTML5 platform.

The Ask: Securing the National Grid

1. Seed Funding

Capital for server architecture and SMS gateway scaling to maintain the high-availability Rural OTP Inclusion Protocol.

2. Pilot Partnerships

Direct access to District Sadar Hospitals to gather real-world validation of the "Doctor-to-Pharmacy" loop under pressure.

3. Regulatory Mentorship

Guidance from a2i and DGDA to achieve formal certification and align perfectly with the National Digital Health Stack.

4. Lab Support

Access to specialized electronic systems labs to stress-test our offline architecture on 10+ year-old hospital PCs.

5. Workplace Hub

Access to a Med-Tech Incubation Center to provide a "War Room" for rapid hardware/software integration and secure stakeholder meetings.